

24NNCV01524 24NNCV01524

County: los-angeles

Division: Civil Law and Motion

Department: E

Hearing date: 2026-07-02

Source URL:

https://www.lacourt.ca.gov/tentativeRulingNet/ui/main.aspx?casetype=civil&dept_date=GLN%2CE%2C07%2F02%2F2026

Source SHA-256: daadd79d5186822d3977940be05de378e18db315eae61df2ef45e7e085016fb4

Case Number: 24NNCV01524 Hearing Date: July 2, 2026 Dept: E

Hearing

Date: 07/02/2026 – 9:30 a.m.

Case No. 24NNCV01524

Trial Date: 06/14/2027

Case Name: RAYMOND TYNDALL, et al. v. CITY OF HOPE, et al.

TENTATIVE

RULING ON 4 MSJs

BACKGROUND

The operative complaint in this action appears to be Plaintiffs' Second Amended Complaint (SAC).

Plaintiffs in this action appear to be:

(1) Raymond Tyndall, individually and as successor in interest to decedent, Susan Tyndall; (2) Kylie Rae Merry, individually and as successor in interest to decedent Susan Tyndall; and (3) Keaton Nicole Meo, individually and as successor in interest to decedent, Susan Tyndall.

Defendants in this action appear to be:

(1) City of Hope; (2) Bradford Kim, M.D.; (3) Kevin Labadie, M.D.; (4) Yanghee Woo, M.D.; (5) Covidien Holding Inc. [Doe 1]; (6) Covidien LP [Doe 2]; (7) Medtronic, Inc. [Doe 3]; (8) Medtronic USA, Inc. [Doe 4]; and (9) DOES 5-100.

The SAC alleges in relevant part that:

(1) Susan Tyndall was diagnosed with adenocarcinoma gastric cancer of the stomach in May 2023 (SAC, ¶ 18.);

(2) Gastric surgery was performed on September 25, 2023, and the surgeon was Dr. Kim, and the assistant surgeons were Dr. Woo and Dr. Labadie (SAC, ¶ 19.);

(3) On September 30, 2023, Ms. Tyndall was discharged despite continuing back pain (SAC, ¶ 21-22.);

(4) A follow-up CT Scan revealed a significant staple line leak, and EGD images showed a gastric leak for a hemircumferential dehiscence of the gastrojejunal anastomosis (SAC, ¶¶ 27-28.);

(5) Dr. Kim rounded on Ms. Tyndall and told her that the stapler used during the September 2023 surgery was “jacked up” (SAC, ¶ 31.);

(6) During the October 9, 2023 Diagnostic Laparoscopy, Robotic Assisted Partial Gastrectomy, Dr. Kim identified the perforation which was specifically located at the transected remnant stomach staple line (SAC, ¶ 33.);

(7) Ms. Tyndall died on August 7, 2024 as a result of complications caused by a failed staple line following gastric cancer surgery (SAC, ¶ 19.).

The SAC alleges nine causes of action.

The first cause of action, labeled as medical malpractice, is alleged against the following Defendants: City of Hope (COH); Dr. Kim; Dr. Labadie; and Dr. Woo.

The second cause of action, labeled as loss of consortium, is alleged against all Defendants.

The third cause of action, labeled as

wrongful death (CCP § 377.60), is alleged against all Defendants.

The fourth cause of action, labeled as product liability, is alleged against the two Covidien Defendants and the two Medtronic Defendants.

The fifth cause of action, labeled as negligence, is alleged against the two Covidien Defendants and the two Medtronic Defendants.

The sixth cause of action, labeled as breach of implied warranty, is alleged against the two Covidien Defendants and the two Medtronic Defendants.

The seventh cause of action, labeled as survivor negligence (CCP § 377.30), is alleged against the following Defendants: City of Hope (COH); Dr. Kim; Dr. Labadie; and Dr. Woo.

The eighth cause of action, labeled as strict liability – failure to warn, is alleged against the two Covidien Defendants and the two Medtronic Defendants.

The ninth cause of action, labeled as breach of express warranty (Cal. Com. Code § 2313), is alleged against the two Covidien Defendants and the two Medtronic Defendants.

Four motions for summary judgment (MSJ) are on calendar for 7/2/2026.

The Court will refer to Defendant's, Bradford Kim, M.D. (Dr. Kim), MSJ as Motion 1 or MSJ 1. [Motion 1 has a Res. I.D. ending in 9009.]

The Court will refer to Defendant's, Yanghee Woo, M.D. (Dr. Woo), MSJ as Motion 2 or MSJ 2. [Motion 2 has a Res. I.D. ending in 9509.]

The Court will refer to Defendant's, Kevin Labadie, M.D. (Dr. Labadie), MSJ as Motion 3 or MSJ 3. [Motion 3 has a Res. I.D. ending in 5695.]

The Court will refer to Defendant's,
City of Hope (COH), MSJ as Motion 4 or MSJ 4. [Motion 4 has a Res. I.D. ending
in 7574.]

[The Court notes that if it is
referring to the moving defendants [the ones who filed the four MSJs] it will
likely refer to them collectively as Movants, or Defendants, whereas if the
Court is collectively referring to non-moving defendants it will refer to them
as Covidien/Medtronic Defendants.]

RELIEF REQUESTED:

Motions 1, 2, and 3 [brought by the Doctor Defendants – Dr.
Kim, Dr. Woo, and Dr. Labadie] move for summary judgment as to Plaintiffs'
Complaint pursuant to CCP § 437c, on the grounds that no triable issue exists
as to any material fact, and Doctor Defendants are entitled to judgment as a
matter of law as to the entire Complaint for the following reasons:

1)

Plaintiffs cannot establish the necessary elements for their cause of action
for Wrongful Death/Medical Malpractice:

a.

Defendant at all times complied with the applicable standard of care.

b.

The conduct of defendant was not a substantial factor in causing any of the
alleged injuries/damages.

2)

Because plaintiffs cannot establish the necessary elements for their cause of
action for Wrongful Death/Medical Malpractice, plaintiff Raymond Tyndall has no
cause of action for Loss of Consortium.

(Mot. 1, 2, & 3, p. 2.)

Motion 4 [brought by City of Hope] moves for summary
judgment as to Plaintiffs' Complaint pursuant to CCP § 437c, on the grounds
that no triable issue exists as to any material fact, and Defendant is entitled

to judgment as a matter of law as to the entire Complaint for the following reasons:

1)

Plaintiffs cannot establish the necessary elements for their cause of action for Wrongful Death/Medical Malpractice:

a. Defendant

physicians at all times complied with the applicable standard of care.

b. The conduct

of defendant physicians was not a substantial factor in causing any of the alleged injuries/damages.

2) Because

plaintiffs cannot establish the necessary elements for their cause of action for Wrongful Death/Medical Malpractice, plaintiff Raymond Tyndall has no cause of action for Loss of Consortium.

3) City of

Hope is not responsible for the conduct of physicians including defendant physicians in this case.

a. Prior to

the procedure in question, decedent Susan Tyndall (hereinafter "decedent") signed "CA General Consent for Treatment" acknowledging that "All physicians and surgeons . . . are independent contractors and are not employees or agents of City of Hope."

(Mot. 4, p. 2.)

PAPERS SUBMITTED

Moving Papers

For all four motions, Movants submitted a: Notice/Motion; Separate Statement; Declaration of Miguel A. Burch; Declaration of Michael B. Van; Proposed Judgment; Proposed Order

The Court notes that each Movant submitted their own set of

moving papers.

Opposing Papers

In opposition, Plaintiffs did not submit opposition papers for each of the four separate MSJs. Instead, Plaintiffs submitted a singular omnibus set that opposed all four of Movants' MSJs.

Those papers were as follows: Omnibus opposition; Omnibus Separate Statement; Compendium of Exhibits; Declaration of Kevin Lowe, M.D.; Declaration of Mark Levin, M.D.; Evidentiary Objections; Declaration of Raymond Tyndall; Declaration of Sohaila Sagheb; Declaration of Geni Bennetts; Proof of Service

Reply Papers

Unlike Movants' moving papers that contained a set of papers for each of the four MSJs, Movants' reply papers are a singular omnibus set of reply papers directed at all four MSJs.

Those papers are: Reply; Objections; Proposed Order

LEGAL

STANDARD

" A party may move for summary judgment in an action or proceeding if it is contended that the action has no merit or that there is no defense to the action or proceeding." (CCP, § 437c(a)(1).)

Code of Civil Procedure section 437c(c)

"requires the trial judge to grant summary judgment if all the evidence submitted, and 'all inferences reasonably deducible from the evidence' and uncontradicted by other inferences or evidence, show that there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law." (Adler v. Manor Healthcare Corp. (1992) 7 Cal.App.4th 1110, 1119.) "The function of the pleadings in a motion for summary judgment is to delimit the scope of the issues; the function of the affidavits or declarations is to disclose whether there is any triable issue of fact within the issues delimited by the

pleadings.” (Juge v. County of Sacramento
(1993) 12 Cal.App.4th 59, 67, citing FPI
Development, Inc. v. Nakashima (1991) 231 Cal. App. 3d 367, 381-382.)

For purposes of motions for summary
judgment and summary adjudication:

A defendant or cross-defendant has met that party's burden
of showing that a cause of action has no merit if the party has shown that one
or more elements of the cause of action, even if not separately pleaded, cannot
be established, or that there is a complete defense to the cause of action.
Once the defendant or cross-defendant has met that burden, the burden shifts to
the plaintiff or cross-complainant to show that a triable issue of one or more
material facts exists as to the cause of action or a defense thereto. The
plaintiff or cross-complainant shall not rely upon the allegations or denials
of its pleadings to show that a triable issue of material fact exists but,
instead, shall set forth the specific facts showing that a triable issue of
material fact exists as to the cause of action or a defense thereto.

(CCP, § 437c(p)(2).)

Courts

“liberally construe the evidence in support of the party opposing summary
judgment and resolve doubts concerning the evidence in favor of that party.” (Dore v. Arnold Worldwide, Inc. (2006)
39 Cal.4th 384, 389.)

To

establish a triable issue of material fact, the party opposing the motion must
produce substantial responsive evidence sufficient to establish a triable issue
of material fact on the merits of the defendant's showing. (Sangster v. Paetkau (1998) 68
Cal.App.4th 151, 163.)

TENTATIVE RULING

Issues regarding Compliance with the
CRC

For the reasons
explained below, the Court is considering DENYING all four of Defendants' MSJs;
however, the Court will hear argument.

As a preliminary matter, the Court notes that Defendants do not move for summary adjudication; Defendants only move for summary judgment as to the entire SAC.

Further, the Court notes that although Plaintiffs' SAC implicates Defendants in four out of the nine causes of action in the SAC – the first cause of action for medical malpractice; the second cause of action for loss of consortium; the third cause of action for wrongful death (CCP § 377.60); and the seventh cause of action for survivor negligence (CCP § 377.30) – Defendants' notice of motion page moves for judgment as to the entire SAC, without specifying which causes of action are at issue, other than by referring to the causes of action as a “cause of action for Wrongful Death/Medical Malpractice” and “Loss of Consortium.”

Or to phrase it differently, Defendants' notice of motion refers to wrongful death/medical malpractice as a singular cause of action despite the fact that Plaintiffs' SAC refers to those as two separate causes of action. Additionally, the SAC labels the seventh cause of action as a claim for survivor negligence, but Defendants' notice of motion does not refer to the claim for survivor negligence.

The Court is considering denying all four of Defendants' MSJs because all four of Defendants' separate statements do not comply with the California Rules of Court (CRC).

CRC, rule 3.1350(h) explains what format a separate statement must follow when the motion is for a motion for summary judgment compared to what format a separate statement must follow when the motion is for a motion for summary adjudication.

Even though Defendants are moving for summary judgment as to the entire SAC, and they don't move for summary adjudication, Defendants do not comply with CRC, rule 3.1350(h) with respect to separate statements for an MSJ.

Under CRC, rule 3.1350(h), with respect to MSJs, the separate statement does not separate the separate statement into separately identifiable issues; the separate statement is done as a singular set of undisputed material facts. Whereas, under CRC, rule 3.1350(h), with respect to

MSAs, the separate statement does separately identify issues. (See CRC, rule 3.1350(h).)

Here, all four of Defendants' separate statements separately identify two issues despite the fact that they are filing an MSJ, not an MSA.

Further, Defendants' separate statements separately identify two issues – "Issue 1-Medical Malpractice" and "Issue 2-Loss of Consortium." (See Def. Sep. Stmts.)

Defendants not following CRC, rule 3.1350(h) is confusing because it is unclear if Defendants think they filed an MSA in the alternative, even though they did not.

Further, to the extent that Defendants tried to follow rule 3.1350(h) with respect to separate statements for MSAs, Defendants did not follow that rule properly.

As explained in CRC, rule 3.1350(d):

(d)

Separate statement in support of motion

(1) The

Separate Statement of Undisputed Material Facts in support of a motion must separately identify:

(A)

Each cause of action, claim for damages, issue of duty, or affirmative defense that is the subject of the motion; and

(B)

Each supporting material fact claimed to be without dispute with respect to the cause of action, claim for damages, issue of duty, or affirmative defense that is the subject of the motion.

(CRC, rule 3.1350(d).)

To the extent that Defendants tried to separately identify issues, Defendants did not comply with CRC, rule 3.1350(d).

Defendants moved for summary judgment, but they only put two issues in their separate statement – “Issue 1-Medical Malpractice” and “Issue 2-Loss of Consortium” – but they failed to separately identify that the third cause of action for wrongful death is at issue in Plaintiffs’ SAC, and they failed to separately identify the seventh cause of action for survivor negligence that is at issue in Plaintiffs’ SAC.

“The separate statement serves two important functions in a summary judgment proceeding: It notifies the parties which material facts are at issue, and it provides a convenient and expeditious vehicle permitting the trial court to hone in on the truly disputed facts.” (Beltran v. Hard Rock Hotel Licensing, Inc. (2023) 97 Cal.App.5th 865, 875.)

“Trial courts⁵ should not hesitate to deny summary judgment motions when the moving party fails to draft a compliant separate statement – and an inappropriate separate statement includes an overly long document that includes multiple nonmaterial facts in violation of the Rules of Court.” (Beltran v. Hard Rock Hotel Licensing, Inc. (2023) 97 Cal.App.5th 865, 876; fn. 5 stating, “In certain instances, particularly before granting summary judgment or adjudication, an opportunity to correct deficiencies in the separate statement may be appropriate. (Parkview Villas Assn., Inc. v. State Farm Fire & Casualty Co., supra, 133 Cal.App.4th at pp. 1215-1216, 35 Cal.Rptr.3d 411; see Rush v. White Corp. (2017) 13 Cal.App.5th 1086, 1100, 221 Cal.Rptr.3d 240.)”).

The confusion does not end with Defendants’ separate statements.

Although four causes of action are at issue with respect to Defendants in Plaintiffs’ SAC, Defendants’ notice of motion moves for summary judgment as to the entire SAC, yet the notice of motion simultaneously explains that Plaintiffs cannot establish two causes of action for “wrongful death/medical malpractice” and “loss of consortium.”

Aside from Defendants’ notice page not referring to the seventh cause of action for survivor negligence that is alleged against them, Defendants’ notice page refers to wrongful death and medical malpractice as a singular cause of action, despite the fact that Plaintiffs’ SAC refers to the first cause of action as a claim for medical malpractice and

the third cause of action as a claim for “wrongful death (CCP § 377.60)”.

In any event, all four of Defendants’ memorandums explain that Plaintiffs cannot establish a cause of action for wrongful death/medical malpractice because Doctor Defendants complied with the standard of care, and there is no evidence that any act or omission of Doctor Defendants caused or contributed to Decedent’s injuries/death, or were a substantial factor to her injuries/death.

To make this argument about wrongful death/medical malpractice, Defendants cite to the alleged elements of medical negligence [duty, breach, causation, and damages] to argue that Plaintiffs cannot establish that Defendants breached the standard of care because Defendants complied with the standard of care, and to argue that Plaintiffs cannot establish causation.

Problematic with Defendants’ argument is that all of their memorandums entirely fail to address how they are meeting their burden with respect to the cause of action for wrongful death.

Defendants cited the elements for medical negligence, explained how Plaintiffs cannot satisfy the elements in a medical negligence action, and simply arrive at the conclusion that Plaintiffs cannot establish claims for both medical negligence and wrongful death.

Or to phrase it differently, it appears as if Defendants assume, without citing to any legal authority, that claims for wrongful death, medical malpractice, and medical negligence are all the same cause of action and all have the same elements.

“A defendant or cross-defendant has met that party’s burden of showing that a cause of action has no merit if the party has shown that one or more elements of the cause of action, even if not separately pleaded, cannot be established, or that there is a complete defense to the cause of action.” (CCP, § 437c(p)(2).)

How do Defendants meet their initial burden in demonstrating that Plaintiffs cannot establish one or more elements of a cause of action for wrongful death if Defendants do not once cite legal authority that explains what the elements are

for a wrongful death claim nor explain what element(s) that Plaintiffs cannot establish?

Because

Defendants' moving papers do not even attempt to explain how they meet their initial burden with respect to the cause of action for wrongful death, it appears that the Court can DENY all four of Defendants' MSJs.

The Court would deny all four of

Defendant's MSJs, for failing to meet their initial burden with respect to the cause of action for wrongful death, because Defendants did not file a motion for summary adjudication with respect to any individual causes of action.

Or to phrase it differently, since

Defendants only filed an MSJ targeted at the entire SAC, it appears as if it doesn't matter if Defendants were able to meet their initial burden with respect to the other causes of action that aren't for wrongful death because Defendants didn't file an MSA directed at specific causes of action.

"A party may move for summary judgment

in an action or proceeding if it is contended that the action has no merit or that there is no defense to the action or proceeding." (CCP, § 437c(a)(1).)

Defendants' MSJ is targeted at the

entire SAC; therefore, since Defendants' MSJ fails with respect to at least the wrongful death claim for Defendants failing to meet their initial burden, Defendants did not establish that the entire action has no merit.

The

Court to hear argument on all the issues discussed in this section

TENTATIVE RULING – SUBSTANTIVE ISSUES

In the event that the Court does resolve

the motions on the issues discussed in the previous section, the Court is inclined to adopt the Tentative Ruling on the substantive issues set out below.

Although Plaintiffs' SAC implicates

Defendants in four out of the nine causes of action in the SAC – the first cause of action for medical malpractice; the second cause of action for loss of

consortium; the third cause of action for wrongful death (CCP § 377.60); and the seventh cause of action for survivor negligence (CCP § 377.30) – Defendants move for summary judgment on the entire SAC by arguing that since Plaintiffs cannot establish the necessary elements in a medical negligence action, Plaintiffs cannot establish the necessary elements in the entire action.

Or to phrase it differently, Defendants assume that all the causes of action directed against them in the SAC hinge on the elements of medical negligence.

In opposition, Plaintiffs do not dispute Defendants' assumption, nor do Plaintiffs attempt to distinguish what the elements are for each of the four causes of action asserted against Defendants.

In any medical malpractice action, the plaintiff must establish: (1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional's negligence. (*Powell v. Kleinman* (2007) 151 Cal.App.4th 112, 122 citing *Hanson v. Grode* (1999) 76 Cal.App.4th 601, 606.)

The first element, standard of care, is the key issue in a malpractice action and can only be proved by expert testimony, unless the circumstances are such that the required conduct is within the layperson's common knowledge. (*San Antonio Regional Hospital v. Superior Court* (2024) 102 Cal.App.5th 346, 350 quoting *Lattimore v. Dickey* (2015) 239 Cal.App.4th 959, 968.)

Both the standard of care and defendants' breach must normally be established by expert testimony in a medical malpractice case.¹ (*Avivi v. Centro Medico Urgente Medical Center* (2008) 159 Cal.App.4th 463, 467; Fn. 1 stating, "In medical malpractice cases where the conduct in question is within the common knowledge of laypersons, expert testimony may be unnecessary, such as cases where the plaintiff can invoke the doctrine of *res ipsa loquitur*.".)

Proving the third element, causation,

also requires competent expert testimony. (San Antonio Regional Hospital v. Superior Court (2024) 102 Cal.App.5th 346, 350 citing Miranda v. Bomel Construction Co., Inc. (2010) 187 Cal.App.4th 1326, 1336.)

An expert is qualified to testify where she possesses special skill or experience in her field so that her testimony is likely to assist the jury in the search for the truth. (San Antonio Regional Hospital v. Superior Court (2024) 102 Cal.App.5th 346, 351 citing Lattimore v. Dickey (2015) 239 Cal.App.4th 959, 969.)

Expert testimony is properly excluded as speculative where the expert lacks expertise over the subject matter of the litigation. (San Antonio Regional Hospital v. Superior Court (2024) 102 Cal.App.5th 346, 352.)

Qualifications other than a license to practice medicine may serve to qualify a witness to give a medical opinion. (San Antonio Regional Hospital v. Superior Court (2024) 102 Cal.App.5th 346, 352 citing People v. Caitlin (2001) 26 Cal.4th 81, 131-132.)

If the expert has disclosed sufficient knowledge of the subject to entitle his or her opinion to go to the jury, the court abuses its discretion by excluding his or her testimony. (Avivi v. Centro Medico Urgente Medical Center (2008) 159 Cal.App.4th 463, 467-468 citing Mann v. Cracchiolo (1985) 38 Cal.3d 18, 39, overruled on other grounds.)

To prevail on a claim for wrongful death, plaintiffs must prove (1) a wrongful act or neglect on the part of one or more persons (that is, negligence) that (2) causes (3) the death of another person. (Musgrove v. Silver (2022) 82 Cal.App.5th 694, 705 citing Norgart v. Upjohn Co. (1999) 21 Cal.4th 383, 390.)

The elements for loss of consortium are: (1) a valid and lawful marriage between the plaintiff and the person injured at the time of the injury; (2) a tortious injury to the plaintiff's spouse; (3) loss of consortium suffered by the plaintiff; and (4) the loss was proximately caused by the defendant's act. (Hahn v. Mirza (2007) 147 Cal.App.4th 740, 746, fn. 2.)

The Court will first address Motions 1
(Dr. Kim), 2 (Dr. Woo), and 3 (Dr. Labadie).

With respect to Motions 1, 2, and 3,
the Doctor Defendants' motions are easy to follow.

Defendants appear to be arguing that
Plaintiffs cannot establish a claim for medical malpractice because Plaintiffs
cannot establish that Defendants breached their duty of care and that
Plaintiffs cannot establish that Defendants are the proximate cause.

Defendants submit expert testimony of
two medical doctors – Dr. Burch and Dr. Mosher.

Dr. Miguel A. Burch, M.D. FACS, M.S, is
a board-certified surgeon duly licensed to practice medicine in the state of
California; he completed his fellowship in Minimally Invasive and Bariatric
Surgery at Cedars-Sinai Medical Center in 2005; and he is familiar with the
standard of care for surgeons in the community of Southern California at all
times relevant to this litigation. (See Miguel A. Burch Decl., ¶¶ 1-5.)

Dr. Michael B. Van Scoy-Mosher, M.D.,
M.A., is board-certified in internal medicine and oncology; is duly licensed to
practice medicine in the state of California; and is familiar with issues of
medical causation including cancer progression at all times relevant to this
litigation. (See Michael B. Van Scoy-Mosher Decl., ¶¶ 1-5.)

Doctor Defendants' motions are easy to
follow because Dr. Burch concludes that Doctors Kim, Woo, and Labadie complied
with the standard of care (UMF 27/Burch Decl. ¶ 31), and Dr. Burch concludes
that no act or omission by Doctors Kim, Woo, and Labadie caused or contributed
to decedent's injuries or death (UMF 33/Burch Decl. ¶ 37).

Dr. Mosher also concludes that no act
or omission by Doctors Kim, Woo, or Labadie caused or contributed to decedent's
injuries or death. (UMF 34/Mosher Decl. ¶ 34.)

Therefore, Doctor Defendants meet their
initial burden in demonstrating that Plaintiffs cannot establish breach and

causation.

Plaintiffs' opposition is baffling and at times incomprehensible.

The Court would imagine that to oppose Defendants' motions, Plaintiffs' experts would have concluded that Doctors Kim, Woo, and Labadie did not comply with the standard of care and that the acts and omissions of Kim, Woo, and Labadie did cause decedent's injuries or death.

Plaintiffs' opposition and Plaintiffs' experts provide no such clarity.

Plaintiffs' opposition goes into all sorts of incomprehensible arguments about how Defendants did not meet their initial burden. Since Plaintiffs' arguments are incomprehensible on this issue, the Court does not find Plaintiffs' arguments on this issue availing.

When Plaintiffs' separate statement and Plaintiffs' experts attempt to dispute Defendants' experts, a vast majority of Plaintiffs' arguments are incomprehensible.

For example, UMF 27 pertains to Dr. Burch's opinion that Doctor Defendants Woo, Kim, and Labadie complied with the standard of care.

Plaintiffs' opposition separate statement disputes Dr. Burch's opinion for several paragraphs within UMF 27 yet doesn't comply with CRC, rule 3.1350(f)(2).

"An opposing party who contends that a fact is disputed must state, on the right side of the page directly opposite the fact in dispute, the nature of the dispute and describe the evidence that supports the position that the fact is controverted. Citation to the evidence in support of the position that a fact is controverted must include reference to the exhibit, title, page, and line numbers." (CRC, rule 3.1350(f)(2).)

With respect to UMF 27, not only do Plaintiffs not cite to evidence on many occasions, but Plaintiffs don't include

reference to the exhibit, page, and line numbers.

Overall, Plaintiffs seem to attempt to attack Defendants' experts, but the attacks on Defendants' experts are incomprehensible.

Ultimately, the Court asks Plaintiffs' counsel the following question – how are any of Plaintiffs' arguments relevant if Plaintiffs' experts can't conclude that Doctor Defendants did not comply with the standard of care or that Doctor Defendants caused the injuries/death?

Plaintiffs' opposition separate statement to UMFs 33 and 34 suffer the same defects that the Court explained with respect to UMF 27.

Additionally, Plaintiffs' opposition submits two experts – Dr. Mark Levin and Dr. Kevin Lowe.

Kevin Lowe, M.D., Ph.D., F.A.C.S. is a physician licensed to practice medicine and is board certified by the American Board of Surgery. He is also a fellow of the American College of Surgeons and certified by the American HepatoPancreatoBiliary Association. He currently practices as a general and cancer surgeon with a focus on gastrointestinal cancer at Indiana University Health in Fort Wayne, Indiana. (Decl. Kevin M. Lowe, ¶¶ 1-3.)

Mark Levin, M.D., is licensed to practice medicine in the states of New York, New Jersey, and Connecticut. He is board certified in Internal Medicine, Medical Oncology, and Hematology by the American Board of Internal Medicine and has practiced clinical oncology and hematology for over three decades. (Decl. Mark Levin, M.D., ¶ 1.)

The Court cannot make any sense of Dr. Levin's Declaration.

Not once does the Court see where Dr. Levin opines/concludes that Doctors Woo, Kim, and Labadie failed to comply with the standard of care or that the Doctor Defendants acts/omissions caused the injuries/death.

The only portion of Dr. Levin's declaration that the Court can understand is that Dr. Levin states "it is my opinion, to a reasonable degree of medical probability, that the staple line failure and its consequences were a substantial contributing cause of Ms. Tyndall's metastatic progression and ultimate death." (Levin Decl., ¶ 17.)

However, with respect to ¶ 17, the Court asks Plaintiffs' counsel – Who, of the moving Defendants, was a substantial contributing cause? Levin explains that the staple line failure and its consequences were a substantial contributing cause, but how does that pertain to moving Doctor Defendants?

The Court is not sure what it is supposed to do with the Levin Declaration when the declaration doesn't explain anything with respect to the Defendants.

The Lowe Declaration is also largely unhelpful to the Court as it does many of the same things that the Levin Declaration does.

For example, ¶ 25 of the Lowe Declaration states that "To a reasonable degree of medical probability, the operative description of deformed staples and large-scale staple line disruption is evidence of failed staple formation and failed construction of the gastric staple line." (Lowe Decl., ¶ 25.)

Again, what is the Court supposed to do with this information when Lowe doesn't explain this conclusion with respect to the Defendants?

The most comprehensible thing that Lowe states is, "Because Defendants and their experts disclaim malfunction, the operative findings support the conclusion that the staple line was improperly created by Dr. Kim, Dr. Labadie and perhaps Dr. Woo. Improper creation of a staple line that results in an anastomotic leak breached the standard of care was breached and caused Susan Tyndall injury." (Lowe Decl., ¶ 45.)

Based on ¶ 45 of the Lowe Declaration, it appears that Plaintiffs successfully demonstrate a dispute of material fact

with respect to breach of the standard of care and causation with respect to Dr. Kim and Dr. Labadie.

Therefore,

Motions 1 and Motion 3 are DENIED because ¶ 45 of the Lowe Declaration demonstrates a dispute of material fact as to breach of the standard of care and causation with respect to Doctors Kim and Doctor Labadie. Although Defendants' reply appears to argue that there is no dispute of material fact with respect to wrongful death because Plaintiffs' experts don't explain how Defendants caused Plaintiffs' death, the Court does not find this argument availing because Defendants did not move for summary adjudication in the alternative. Or to phrase it differently, Defendants only moved for summary judgment as to the entire SAC. At the very least, Plaintiffs demonstrated a dispute of a material fact with respect to the medical malpractice/negligence action.

With

respect to Motion 2,

pertaining to Dr. Woo, the Court fails to see how Plaintiffs demonstrated a dispute of a material fact as to breach of the standard of care and causation. Dr. Lowe states that the staple line was improperly created by Dr. Kim, Dr. Labadie, and "perhaps Dr. Woo." (Lowe Decl. ¶ 45.) The Court has no idea what "perhaps Dr. Woo" means. How can Plaintiffs demonstrate a dispute of material fact as to standard of care and causation with respect to Dr. Woo if Lowe only concludes "perhaps Dr. Woo?" The Court is inclined to GRANT summary judgment in favor of Dr. Woo, i.e. grant Motion 2, because the Court fails to see how Plaintiffs' experts rendered a conclusion with respect to Dr. Woo.

With respect to Motion 4, which is

brought by the Hospital Defendant, City of Hope, both parties' arguments are incomprehensible.

City of Hope first argues that because

Doctor Defendants complied with the standard of care and because Doctor Defendants were not a substantial factor in causing injuries/damages, Plaintiffs cannot establish the necessary elements for their causes of action.

City of Hope's argument here makes no

sense. First, there is a dispute of material fact with respect to breach and causation with respect to Doctor Defendants Kim and Labadie; therefore, City of Hope's own argument would defeat its own theory.

Second, City of Hope argues that it is not responsible for the conduct of the physicians in this case because Susan Tyndall signed a CA General Consent for Treatment acknowledging that "All physicians and surgeons...are independent contractors and are not employees or agents of City of Hope." (UMF, 9, Def. Ex. A, p. 14-16.) City of Hope also points to how the pre-op checklist on page 16 of Exhibit A indicates that the patient was alert/oriented.

City

of Hope cites to the case of *Wicks v. Antelope Valley Healthcare Dist.* (2020) 49 Cal.App.5th , 866 which states, "It is well established in California that a hospital may be liable for the negligence of physicians on the staff, unless the hospital has clearly notified the patient that the treating physicians are not hospital employees and there is no reason to believe the patient was unable to understand or act on the information. This rule is founded on the theory of ostensible agency." (Id. at p. 882.)

Typically,

a corporation may be liable for employee misconduct under a respondeat superior theory. (*C.R. v. Tenet Healthcare Corp.* (2009) 169 Cal.App.4th 1094, 1110.)

At

common law, an employer could be held vicariously liable for its employee's torts if the torts were committed in the scope of employment. (*Presbyterian Camp & Conference Centers, Inc. v. Superior Court* (2019) 42 Cal.App.5th 148, 156.) The Legislature codified this common law rule nearly 150 years ago. (Id. citing Civ. Code, §2388.)

Here, the Court to hear argument with respect to Motion 4.

Plaintiffs

do not dispute that Tyndall signed the General Consent.

Instead,

Plaintiffs object because Defendants did not produce the consent in discovery.

Problematic

with Plaintiffs' argument about Defendants not producing the consent in discovery is that Plaintiffs do not cite to any legal authority that states that Defendants cannot rely on the consent form in a motion for summary judgment just because it wasn't produced in discovery.

Instead

Plaintiffs' opposition and separate statement goes into different duties that the hospital had within the meaning of the 21 C.F.R. § 803.3(d) and having a mandatory duty to report certain things.

Plaintiffs

also submit the declaration of Geni Bennetts, M.D., a Doctor of Medicine specializing in the area of Hospital Administration, Quality Assurance, Risk Management, Medical Billing Advocacy, staff recruitment and training, cost analysis and budget development, strategic planning, resource management, performance management, client relationship development, and project management and implementation.

Bennetts

concludes that "It is therefore my opinion, to a reasonable degree of medical probability, that City of Hope's failures in quality assurance, adverse-event investigation, patient-safety oversight, and physician governance fell below the applicable standard of care and constituted a substantial contributing factor in the failure to identify, investigate, and appropriately respond to the circumstances that resulted in Ms. Tyndall's injuries and subsequent death." (Bennetts, Decl. ¶ 24.)

Plaintiffs'

opposition argues that a reasonable jury could conclude that City of Hope never investigated and swept Susan's complications under the rug because City of Hope, through Drs. Kim and Woo knew all along that there was no stapler malfunction and that Susan's surgical complications resulted from incompetence care, not device failure.

The Court to hear argument on Motion 4, because as the reply points out, Plaintiffs' SAC did not allege a duty under the C.F.R., nor did Plaintiffs allege that the City of Hope was negligent for failing to maintain appropriate quality assurance and patient-safety and physician oversight.

“The function of the pleadings in a motion for summary judgment is to delimit the scope of the issues; the function of the affidavits or declarations is to disclose whether there is any triable issue of fact within the issues delimited by the pleadings.” (Juge v. County of Sacramento (1993) 12 Cal.App.4th 59, 67, citing FPI Development, Inc. v. Nakashima (1991) 231 Cal. App. 3d 367, 381-382.)

The Court has no idea how the issues that Plaintiffs discuss in opposition have anything to do with the pleadings in the SAC. If they do, Plaintiffs should be prepared to point to where in the SAC such allegations exist.

Plaintiffs’ objections to Dr. Burch and Dr. Mosher are OVERULED.

Defendants’ objections to Dr. Lowe are OVERRULED.

Defendants’ objections to Dr. Levin are OVERRULED.

The Court will hear argument about Defendants’ objections to Dr. Bennetts.